

CHARLOTTE HUNGERFORD HOSPITAL

Pulmonology Department | 540 Litchfield Street, Torrington, CT 06790

OUTPATIENT PULMONOLOGY VISIT SUMMARY

Patient	DOB	Visit Date	Provider	Visit Type
Turner, Megan E.	06/06/1952	February 18, 2026	Nina B. Chowdhury, MD	Post-Exacerbation Follow-up

REASON FOR VISIT

Post-exacerbation follow-up after January 2026 COPD exacerbation with acute bronchitis. Review nocturnal supplemental oxygen initiated December 2025. Spirometry reassessment.

VITAL SIGNS

BP	Heart Rate	Resp. Rate	O2 Sat (room air)	O2 Sat (exercise)
130/76 mmHg	70 bpm	17/min	94%	89% (6-min walk)

SPIROMETRY (POST-BRONCHODILATOR)

Measurement	This Visit	Feb 2025 (Baseline)	Change
FEV1 (L)	1.41	1.48	-0.07 (-5%)
FVC (L)	2.55	2.68	-0.13 (-5%)
FEV1/FVC	0.55	0.55	Stable
FEV1 % Predicted	64%	67%	Slightly reduced, consistent with post-exacerbation

ASSESSMENT AND PLAN

- COPD GOLD Stage II, post-exacerbation recovery: Patient has clinically recovered from the January 2026 acute bronchitis exacerbation. Spirometry is stable versus baseline. Continue tiotropium bromide 18 mcg daily and albuterol rescue inhaler PRN.
- Nocturnal supplemental oxygen: Patient reports improved sleep quality since starting 1L O2 via nasal cannula in December 2025. Nocturnal O2 saturation recording from January shows mean SaO2 of 93%, with no prolonged desaturation events. Continue at 1L/min nightly. Next review at October 2026 visit.
- Exacerbation prevention: Reviewed written action plan. Instructed patient to begin contacting this office if rescue inhaler use exceeds 2 days per week or if mucopurulent sputum develops. Patient understands. David Turner (son and POA) was present and confirmed.
- Next pulmonology visit: October 21, 2026.

Dictated and signed: Nina B. Chowdhury, MD | February 18, 2026

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